TrustVet

PET MEDICAL RECORD
HỒ SƠ Y KHOA THÚ CƯNG

INFORMATION

HỌ VÀ TÊN CHỦ/OWNER NAME: Nguyễn Thị Mỹ Lệ

TÊN THÚ/ NAME: NHÍ ______________________ **LOÀI/SPECIES:** CHÓ (~ 2019) _______________

GIỐNG/BREED: Poodle (8.6kg)________________ **GIỚI TÍNH/SEX:** ĐỰC _______ ☐ **triệt sản**

1. **Thông tin chung:**

Chế độ ăn / Diet	Vaccination	Xổ giun / Deworming
Trưa và tối: tim/heo/gà + rau củ luộc		

2. **Triệu chứng / Clinical signs**:

Đi khám Sức khoẻ tổng quát và Xét nghiệm máu, ăn uống bình thường, giảm linh hoạt.

Ngưng thuốc co giật sẽ lên cơn ngay.

3. **Chẩn đoán/Diagnosis:**

   ĐỘNG KINH, CO GIẬT, SUY GAN, THIẾU MÁU NHẸ

4. **Chẩn đoán CLS:** Men gan ALT/ALKP không đo được, GGT cao, Hct thấp nhẹ

5. **Điều trị khuyến cáo:**

- Thuốc uống hỗ trợ gan (silymarin) uống 2 lần/ngày, trước/khi ăn
- Thuốc uống lợi mật (Ursoacid) uống 1 lần/ngày, sáng – khi ăn
- Liên hệ với bác sĩ thú y địa phương để theo dõi chỉ số ALT/GGT/ALKP mỗi 2 tuần và truyền dịch/ điều trị trợ sức nếu có biến chứng khác
- Tái khám 07/06 (xét nghiệm chỉ số gan)

PET ECHOCARDIOGRAM REPORT
KẾT QUẢ SIÊU ÂM TIM

TrustVet

INFORMATION

HỌ VÀ TÊN CHỦ - OWNER NAME: Nguyễn Thị Mỹ

TÊN THÚ - PET NAME: NHÍ **LOÀI - SPECIES:** Chó

GIỐNG - BREED: **Poodle** **NGÀY SINH - D.O.B:** 2019 **GIỚI TÍNH - SEX:** Đực (chưa TS)

ĐIỂM CƠ THỂ - BCS: 6/9 **NHỊP TIM – HR: 79** bpm **HUYẾT ÁP – BP: 184/87 – 134/78**

INTERPRETATION

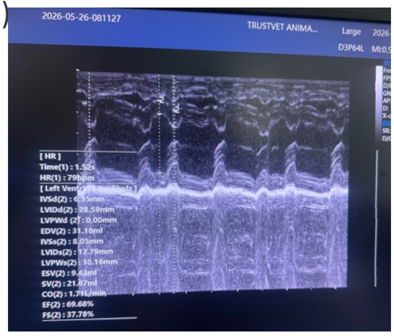
Aorta root diameter:

Left Atrium:

Ao/LA 0.61

M-mode measurement:

Wall	Chamber	Systolic Function
IVSd : 6.35 (6.5)	LVIDd : 28.59 (11.9)	EF : 69 (68.9%)
IVSs : 8.05 (7.3)	LVIDs: 17.7 (18.67)	FS: 37.8 (36.1%)
LVPWs : 10.16 (6.35)		



Mass

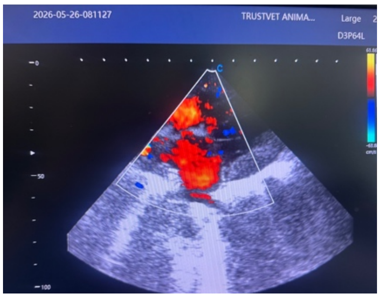
Linear:

Pericardium: chưa phát hiện bất thường

Aorta: doppler hỗn hợp

Vena Cava: chưa phát hiện bất thường

Other findings:



CONCLUSION

Chức năng tim tốt. Chỉ số xét nghiệm gan cao vượt ngưỡng nguy hiểm **Ngày 26/05/2026**

Khuyến cáo dùng thức ăn hỗ trợ Hepatic hỗ trợ gan

ThS. BSTY. Diệp Ngọc Trúc

TrustVet

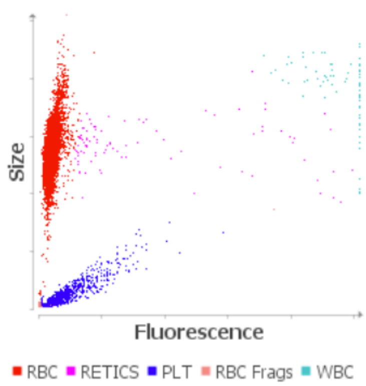
Client: (PE250416003)
Patient Name: NHI
Species: Canine
Breed: Poodle

Gender: Male
Weight: 8.60 kgs
Age: 5 Years
Doctor: DVM. DIEP NGOC TRUC

ProCyte Dx (May 26, 2026 11:17 AM)

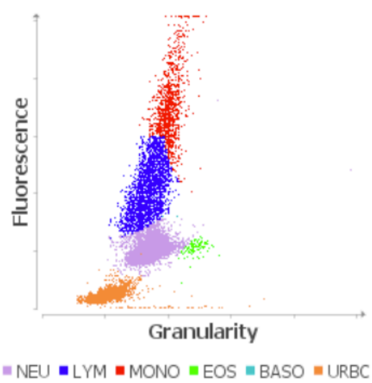
Test	Results	Reference Interval	LOW / NORMAL / HIGH	5/19/25 1:45 PM
RBC	4.88 M/µL	5.65 - 8.87	LOW	7.82 M/µL
HCT	32.1 %	37.3 - 61.7	LOW	50.6 %
HGB	11.3 g/dL	13.1 - 20.5	LOW	18.3 g/dL
MCV	65.8 fL	61.6 - 73.5		64.7 fL
MCH	23.2 pg	21.2 - 25.9		23.4 pg
MCHC	35.2 g/dL	32.0 - 37.9		36.2 g/dL
RDW	15.5 %	13.6 - 21.7		17.9 %
%RETIC	0.4 %			0.2 %
RETIC	19.5 K/µL	10.0 - 110.0		14.9 K/µL
RETIC-HGB	23.6 pg	22.3 - 29.6		25.1 pg
WBC	14.32 K/µL	5.05 - 16.76		13.63 K/µL
%NEU	70.6 %			73.9 %
%LYM	19.6 %			19.3 %
%MONO	8.9 %			3.8 %
%EOS	0.9 %			3.0 %
%BASO	0.0 %			0.0 %
NEU	10.11 K/µL	2.95 - 11.64		10.07 K/µL
LYM	2.80 K/µL	1.05 - 5.10		2.63 K/µL
MONO	1.28 K/µL	0.16 - 1.12	HIGH	0.52 K/µL
EOS	0.13 K/µL	0.06 - 1.23		0.41 K/µL
BASO	0.00 K/µL	0.00 - 0.10		0.00 K/µL
PLT	166 K/µL	148 - 484		284 K/µL
MPV	14.4 fL	8.7 - 13.2	HIGH	12.9 fL
PDW	16.0 fL	9.1 - 19.4		12.3 fL
PCT	0.24 %	0.14 - 0.46		0.37 %

RBC Run



1. Anemia without reticulocytosis - Likely non-regenerative anemia; consider pre-regenerative anemia.

WBC Run



1. Monocytosis - Consider inflammation (if lymphopenia, consider glucocorticoid response).

IDEXX LABORATORIES

845 Nguyen Kiem Str., Ward 3, Go Vap Dist., HCMC (084) 0949911186 www.trustvet.vn

Client: MY LE (TV 1109)
Patient Name: NHI
Species: Canine
Breed:

Gender: Male
Weight:
Age: 7 Years
Doctor:

Test	Results	Reference Interval	LOW / NORMAL / HIGH
Catalyst One (May 26, 2026 2:45 PM)			
GLU	96 mg/dL	74 - 143	
CREA	0.6 mg/dL	0.5 - 1.8	
BUN	12 mg/dL	7 - 27	
BUN/CREA	18		
PHOS	4.8 mg/dL	2.5 - 6.8	
CA	9.0 mg/dL	7.9 - 12.0	
TP	7.6 g/dL	5.2 - 8.2	
ALB	2.7 g/dL	2.3 - 4.0	
GLOB	4.8 g/dL	2.5 - 4.5	HIGH
ALB/GLOB	0.6		
ALT	--.-- U/L	10 - 125	
ALKP	> 2000 U/L	23 - 212	HIGH
GGT	108 U/L	0 - 11	HIGH
TBIL	0.3 mg/dL	0.0 - 0.9	
CHOL	211 mg/dL	110 - 320	
AMYL	884 U/L	500 - 1500	
LIPA	545 U/L	200 - 1800	
PHBR	27.8 µg/mL		

PHBR
Subtherapeutic <15 ug/mL
Therapeutic 15-45 ug/mL
Ideal 20-30 ug/mL
Possibly Toxic >30 ug/mL

In most patients, steady state is achieved after two to three weeks of consistent dosing with phenobarbital. Once steady state is achieved, timing of sample collection is not important in more than 90% of patients. However, there can be variability of the phenobarbital half-life in a small percentage of patients. Therefore, if toxicity is suspected, a peak sample (4-5 hours post-pill) may be helpful, and if breakthrough seizures are occurring and inadequate dosing is suspected, a trough level (collected immediately prior to the next dose) may be helpful. Therapeutic monitoring should be performed after two to four weeks of consistent dosage following initiation of treatment or dosage change to allow most patients to achieve a relatively steady state. Patients on lower doses (mg/kg) may take longer to come to steady state. Consistent timing of sampling remains important for comparison across time as there may still remain some fluctuation throughout the day, especially for patients receiving higher doses. Monitoring should then be repeated at a minimum of every six months thereafter, depending on clinical response.

Test	Results	Reference Interval
Catalyst One (May 26, 2026 2:28 PM)		
Na	151 mmol/L	144 - 160
K	4.2 mmol/L	3.5 - 5.8
Na/K	36	
Cl	117 mmol/L	109 - 122

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Client: (PE250416003)	Gender: Male
Patient Name: NHI	Weight: 8.60 kgs
Species: Canine	Age: 5 Years
Breed: Poodle	Doctor: DVM. DIEP NGOC TRUC

Test	Results	Reference Interval	LOW	NORMAL	HIGH	5/19/25 2:11 PM
Catalyst One (May 26, 2026 11:33 AM)						
TT4	1.9 µg/dL	1.0 - 4.0				3.1 µg/dL

Diagnostic Interpretation for TT4

< 1.0 µg/dL	Low
1.0 - 2.0 µg/dL	Low Normal
1.0 - 4.0 µg/dL	Normal
> 4.0 µg/dL	High
2.1 - 5.4 µg/dL	Therapeutic

Dogs with no clinical signs of hypothyroidism and results within the normal reference range are likely euthyroid. Dogs with low T4 concentrations may be hypothyroid or "euthyroid sick". Occasionally, hypothyroid dogs can have T4 concentrations that are low normal. Dogs with clinical signs of hypothyroidism and low or low normal T4 concentrations may be evaluated further by submission of freeT4 (fT4) and canine TSH. A high T4 concentration in a clinically normal dog is likely a variation of normal; however elevations may occur secondary to thyroid auto antibodies or rarely thyroid neoplasia. For dogs on thyroid supplement, acceptable 4-6 hour post pill total T4 concentrations generally fall within the higher end or slightly above the reference range.

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